

Rapid Response Team (RRT) Policy

Early Warning Recognition and Clinical Escalation

Regulation: 42 CFR §482.13(e) | §482.21

POLICY OVERVIEW

Rapid Response Teams (RRT), also known as Medical Emergency Teams (MET), are deployed to bedside patients showing signs of clinical deterioration before a full cardiac or respiratory arrest occurs. RRT programs have been shown to reduce in-hospital cardiac arrests and improve patient outcomes.

KEY REQUIREMENTS

1 Activation Criteria

Any staff member, patient, or family member may activate the RRT without fear of reprisal. Clear activation criteria must be posted and communicated, including: acute change in heart rate, respiratory rate, blood pressure, oxygen saturation, mental status, or concern that the patient 'just doesn't look right.'

2 RRT Composition and Response Time

The RRT must include a minimum of: a critical care nurse or advanced practice provider and a respiratory therapist. A physician must be available by phone or in person. The team must respond to the bedside within 5 minutes of activation.

3 RRT Assessment and Intervention

The RRT must conduct a rapid systematic assessment using a standardized framework (SBAR, I-SBAR-R). Interventions may include: airway management, IV access, fluid resuscitation, medication administration, and arrangement of ICU transfer if needed.

4 Documentation Requirements

All RRT activations must be documented including: time of activation, team members present, clinical findings, interventions performed, and patient disposition. A standardized RRT documentation tool must be used.

5 Afferent and Efferent Limb Monitoring

RRT programs must monitor both the afferent limb (early warning system performance, including the number of hours between deterioration and RRT activation) and the efferent limb (RRT response effectiveness and outcomes). Data must drive program improvement.

COMPLIANCE NOTES

Family-activated RRT (allowing patients/families to call the RRT directly) is an emerging standard that supports patient-centered care and patient rights. RRT data including activation rates, outcomes, and ICU transfer rates must be reviewed by the quality committee.

COMMON SURVEY CITATIONS

Supported under §482.13(e) – Safe environment, and §482.21 – QAPI monitoring of RRT outcomes and code blue rates.